A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

22 April 2026

Professor Hang Quach

Dear Professor Hang Quach,

Re: Mr Michael Debinski
DOB: 1956-04-11
Address: 34 Delbridge Street, FITZROY NORTH, VIC, 3068

- **I would be grateful if you could urgently see Michael for haematological assessment and management of likely new diagnosis of Lymphoma (PET CT ATTACHED)Patient phone number : 0412262431**

I would be grateful if you could urgently see Michael Debinski, a 70-year-old gentleman, regarding investigations concerning for a lymphoproliferative disorder. Michael was initially noted to have prominent, necrotic mesenteric lymph nodes on CT imaging, which prompted further evaluation with a PET/CT scan.

The PET/CT performed has confirmed the presence of an enlarged nodal conglomerate centrally within the mesentery, measuring approximately 4.2 x 4.6 cm. This conglomerate demonstrates intense metabolic activity with an SUV max of 5.7. While there are multiple other mesenteric lymph nodes present, they do not currently show significant FDG uptake.

Importantly, the scan has also identified evidence of disease above the diaphragm. There is increased metabolic activity in the cervical region, specifically involving a left level IIB cervical lymph node (SUV max 5.6) and a left level III node (SUV max 2.6). Given the distribution and the metabolic intensity of these findings, the clinical suspicion for lymphoma is high. Curiously though his LDH and FBE are unremarkable.

Incidental findings on the scan include bilateral renal lesions; the left-sided lesion was previously characterised as a renal cell carcinoma, while an 18 mm exophytic lesion on the right remains indeterminate. He is under the care of a Urologist Dr Uri Hanegbi, although Uri has not been aware of possible RCC, I have asked Michael to have a haematological assessment in the 1st instance.

All appointments and enquires
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Endoscopy locations
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There was no evidence of metabolically active primary neoplastic disease within the lungs or mediastinum.

The PET/CT findings are highly suggestive of a nodal lymphoma involving both mesenteric and cervical sites. Given the accessibility of the cervical lymphadenopathy, the radiologist has recommended an ultrasound-guided biopsy of the left level IIB node to obtain histological confirmation.

I am hoping you can see Michael for this. If there is delay Im sure and confident he can be seen by your excellent colleagues in your practice. PET (SVPHM) below. Please feel free to contact me if you require any further information

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Andrew Osborne

Investigation Results:

EXAMINATION:
F-18 FDG PET/CT

Clinical Indications:
70-year-old male with suspected lymphoma. Noted to have prominent necrotic mesenteric lymph nodes.

Previous Imaging:
Comparison made with CT chest, abdomen/pelvis (dated 02/04/2026). The CT abdomen/pelvis reports dated between 21/02/2024 and 31/07/2025 were also reviewed.

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Technique:
Imaging was acquired from the vertex to the upper thighs with a PET/CT scanner (GE Discovery 710 with time of flight) after the injection of 328 MBq of F-18 Fluorodeoxyglucose (FDG). A contemporaneous low dose non diagnostic CT was performed for anatomic correlation and attenuation correction only. Uptake time was 60 mins. BSL at the time of injection was 6.2 mmol/L.

Findings:

Nodal findings:
There is intense metabolic activity (SUV max 5.7), corresponding to an enlarged nodal conglomerate measuring 4.2 x 4.6 cm in size, located centrally within the mesentery. There are several other prominent nonenlarged mesenteric lymph nodes, that are increased in number when compared to normal. The smaller mesenteric nodes do not demonstrate increased FDG uptake.

There is increase in the number of cervical lymph nodes when compared to normal. There is increased metabolic activity corresponding to a left level IIB cervical lymph node (SUV max 5.6, and a left level III cervical lymph node (SUV max 2.6).

The aforementioned findings are concerning for nodal sites of lymphoma.

Extranodal findings:
No features to suggest the presence of metabolically active extranodal sites of lymphoma. Normal pattern of FDG uptake in the liver relative to the spleen. Physiological FDG uptake is demonstrated in the imaged marrow. No FDG avid pulmonary nodule/mass or areas of airspace opacity/consolidation.

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No features to suggest the presence of a metabolically active primary neoplastic process or metastases involving the lungs, mediastinum, intra-abdominal viscera, omentum/peritoneum, or head/neck.

Further findings:

Increased FDG uptake between the L5 and S1 spinous processes, is probably due to Baastrup's disease.
Age-related prostatomegaly.
Small subcutaneous nodule measuring 12 mm in size in the right posterior upper arm demonstrating mild metabolic activity (SUV max 2.3). This is felt to be benign in nature rather than representing an extranodal sites of lymphoma.
Moderate vascular calcification of the left anterior descending artery.
Exophytic lesion measuring 14 mm in size arising from the inferior-mesial cortex of the left kidney, corresponds to the lesion characterised as a renal cell carcinoma on prior CT examinations. There is an exophytic lesion measuring 18 mm in size arising within the posterior cortex of the midpole of the right kidney, there has been characterised as indeterminate on prior CT examinations. Coexisting bilateral simple renal cysts are present.
Focal area of increased FDG uptake (SUV max 7.9), corresponding to the left maxillary alveolar margin, could be due to periodontal disease.
Symmetrical pattern of increased FDG uptake involving the tongue base/ oropharynx, and slight asymmetrical increased FDG uptake involving the right posterior wall/posterolateral wall of the nasopharynx, is felt to be physiological. No structural abnormality is appreciated at these locations on the low-dose CT examination.
Increased FDG uptake in the large intestine, is physiological.
Mucous retention cyst within the right maxillary sinus.
Pleural-based lipoma overlying the posterior segment of the right lung upper lobe

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Opinion:
Intense metabolic activity corresponding to an enlarged nodal conglomerate located centrally within the mesentery. There are several other smaller mesenteric lymph nodes, that are increased in number when compared to normal. In addition on the background of an overall increase in the number of cervical lymph nodes, increased FDG uptake is present corresponding to left level IIb and III cervical lymph nodes. The aforementioned findings are concerning for nodal sites of lymphoma. No features to suggest the presence of metabolically active extranodal sites of lymphoma.

For histological confirmation, ultrasound-guided sampling of the left level IIB lymph node should be considered.

Reported by:
Dr Ganesh Ramaseshan
Radiology Consultant

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